



Operative Procedure:

Radical cystectomy with urinary diversion, ileal loop.

Pre-Operative Diagnosis:

Bladder CA

Specimen Received:

- A: Right distal ureter (FS)
- B: Left distal ureter (FS)
- C: Right ovary
- D: Bladder, fallopian tube, vagina and left ovary
- E: Right lower pelvic lymph nodes
- F: Left upper pelvic lymph nodes
- G: Left lower pelvic lymph nodes

Final Pathologic Diagnosis:

- A. Ureter, right distal, biopsy for frozen section diagnosis:
Segment of ureter, negative for malignancy
Frozen section diagnosis confirmed
- B. Ureter, left distal, biopsy for frozen section diagnosis:
Segment of ureter, negative for malignancy
Frozen section diagnosis confirmed
- C. Ovary and fallopian tube, right, salpingoophorectomy:
Ovary with no significant pathologic change
Fallopian tube with Walthard cell rests
- D. Urinary bladder, radical cystectomy:

Tumor histologic type: Urothelial carcinoma

Variant histology (%): Not identified

Tumor size: 3.3 cm

Additional dimensions: 2.4 x 1.2 cm (largest anterior wall mass
predominantly papillary)

Grade (WHO 2004): High

Tumor configuration: Papillary (anterior wall mass) and focal residual
invasive carcinoma (posterior wall mass)

Associated epithelial lesions: Urothelial dysplasia

Microscopic tumor extension: Tumor focally invades deep muscularis propria
(see Note)

Lymph-vascular invasion: Not identified

Margins

Urethral: Free of neoplasm

Ureteral: Free of neoplasm (frozen section diagnoses)

ICD-O-3

Carcinoma, urothelial NOS
812013

Site ~~C67.9~~
Bladder NOS C67.9

path
Bladder, overlapping lesion
(anterior/posterior wall) C67.8

gr 8/22/13

confirmed)

Paravesical soft tissue: Free of neoplasm

Other (specify): Not applicable

Additional pathologic findings: Pseudosarcomatous myofibroblastic proliferation, consistent with previous transurethral resection site reaction involving posterior wall with focal residual invasive carcinoma

Grossly inconspicuous microscopic focus of invasive urothelial carcinoma involving the right lateral wall

Paravesical fat necrosis

Paravesical lymph nodes: None

All lymph nodes: 0 positive of 22 lymph nodes (all parts)

Size of largest lymph node metastasis: Not applicable

Extranodal extension (location): Not applicable

Pathologic stage (2010) pT2b pN0 pM-not applicable

Ovary and fallopian tube, left, salpingoophorectomy:

No significant pathologic change

Vaginal mucosal, partial resection:

No significant pathologic change

E. Lymph nodes (9), right lower pelvic, regional resection:

Nine lymph nodes with no pathologic change (0/9)

F. Lymph nodes (12), left upper pelvic, regional resection:

Twelve lymph nodes with no pathologic change (0/12)

G. Lymph node (1), left lower pelvic, regional resection:

One lymph node with no pathologic change (0/1, specimen submitted entirely for microscopic examination)

The examination of this case material and the preparation of this report were performed by the staff pathologist.

Note:

The anterior wall mass is predominantly papillary without invasion. The posterior wall mass is composed predominantly of a pseudosarcomatous myofibroblastic proliferation that extends deep into the muscularis propria, likely the result of the previous transurethral resection. Nests of atypical cells are present deep in this myofibroblastic proliferation. In order to confirm the epithelial nature of these cells and distinguish them from the reactive process, immunohistochemical staining is performed. Immunohistochemical stains highlight numerous nests of invasive urothelial carcinoma cells, most of

which were light microscopically inconspicuous (positive for p63 and cytokeratin cocktail). The tumor is negative for GATA3, which is relatively specific but not entirely sensitive for urothelial carcinoma. Controls stain appropriately. Multiple additional sections of this area are also examined, revealing similar findings.

Intraoperative Consult Diagnosis:

FSA: Right distal ureter:

Negative for malignancy.

FSB: Left distal ureter:

Negative for malignancy.

F/S TAT:

Gross Description:

A. Received without fixative labeled and "right distal ureter" is a 0.5 x 0.4 cm tan, tubular soft tissue with minimal adherent fat, submitted in toto for frozen section microscopy, now in cassette A.

B. Received without fixative labeled and "left distal ureter" is a 0.5 x 0.5 cm tan, tubular soft tissue with minimal adherent fat, submitted in toto for frozen section microscopy, now in cassette B.

C. Received in formalin, labeled and "right ovary" is a 5.0 x 0.4 cm purple-pink, fimbriated fallopian tube with attached 3.0 x 0.9 x 0.9 cm tan, lobulated intact ovary. The fallopian tube on sectioning contains a pinpoint lumen. The ovary on sectioning is tan, heterogeneous with a 0.3 cm smooth walled uniloculated cyst. Representative sections are submitted in one cassette.

D. Received in formalin, labeled and "bladder, fallopian tube, left ovary, vagina" is a 5.0 x 4.2 x 3.8 cm urinary bladder with attached bilateral segments of distal ureter (each 1.5 x 0.4 cm), 2.9 x 1.0 cm urethra, 4.8 x 4.8 cm tan-pink, wrinkled irregular anterior wall of vagina and a moderate amount of attached fat. Involving the anterior wall of the bladder is a 3.3 x 2.4 x 1.2 cm tan-pink, irregular, papillary appearing, well delineated mass that comes to within 2.7 cm from the urethral margin. This mass on sectioning is tan-pink, soft, friable with no definite invasion, coming to within 0.9 cm from the inked soft tissue margin. On the posterior wall of the bladder is a 3.2 x 1.7 cm tan-pink, irregular ulcerated, well delineated mass that comes to within 5.5 cm from the urethral margin. This mass on sectioning, the uninvolved bladder mucosa is tan-pink and wrinkled. No involvement of the vaginal mucosa is present. No lymph nodes are identified in the attached fat.

Received in the same container is a 3.8 x 0.5 cm purple-pink, fimbriated fallopian tube with attached 2.0 x 0.9 x 0.9 cm tan, lobulated intact ovary. The fallopian tube on sectioning contains a pinpoint lumen. The ovary on sectioning is tan and heterogeneous with no discrete lesions.

Representative sections are submitted as follows:

- D1 urethral margin taken en face;
- D2 right lateral vaginal mucosal margin taken en face;
- D3 left lateral vaginal mucosal margin taken en face;
- D4 superior vaginal mucosal margin taken en face;
- D5-8 mass anterior wall of bladder to inked soft tissue margin;
- D9 mass posterior wall of bladder to inked soft tissue margin;
- D10-11 mass posterior wall of bladder;
- D12 vaginal wall;
- D13 uninvolved right lateral wall of bladder;
- D14 uninvolved left lateral wall of bladder;
- D15 uninvolved dome of bladder;
- D16 trigone;
- D17 ovary and fallopian tube;
- D18-19 perivesicular fat for possible lymph node capture;
- D20-21 peri dome fat for possible lymph node capture.
- D22-29 remainder of posterior wall mass

E. Received in formalin, labeled and "right lower pelvic lymph nodes" is a 6.9 x 4.3 x 1.8 cm aggregate of tan, lobulated irregular fatty tissue. Sectioning reveals multiple tan-pink, rubbery lymph nodes up to 3.5 x 2.0 x 1.0 cm.

Representative sections are submitted as follows:

- E1 multiple whole lymph nodes;
- E2 one whole lymph node bisected;
- E3 one whole lymph node bisected;
- E4 one whole lymph node bisected;
- E5 one whole lymph node bisected;
- E6 one whole lymph node trisected;
- E7-9 one whole lymph node step sectioned.

F. Received in formalin, labeled and "left upper pelvic lymph nodes" is a 5.7 x 5.4 x 2.8 cm aggregate of tan, lobulated irregular fatty tissue. Sectioning reveals multiple tan-pink, rubbery lymph nodes up to 4.8 x 1.4 x 1.0 cm.

Representative sections are submitted as follows:

- F1-2 multiple whole lymph nodes;
- F3 one whole lymph node bisected;
- F4-5 one whole lymph node bisected;
- F6-11 one whole lymph node step sectioned.

G. Received in formalin, labeled and "left lower pelvic lymph nodes" is a 4.2 x 3.7 x 1.8 cm aggregate of tan, lobulated irregular fatty tissue. Sectioning reveals a tan, lobulated cut surface with no discrete lymph nodes. The specimen is entirely submitted in eight cassettes.

Please note, all fat is placed in a clearing fixative prior to searching for

lymph nodes.

Microscopic Description:

The final diagnosis of each specimen incorporates the microscopic examination findings.

END OF REPORT

Taken: DOB: (Age: Gender: F

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary		
Case is (circle):	QUALIFIED	DEQUALIFIED
Reviewer Initials	MB	8/9/13